

CITATION: Slaght v. Phillips and Wicaartz, 2010 ONSC 6464
COURT FILE NO.: CV-07-109
DATE: 2010-12-09

ONTARIO
SUPERIOR COURT OF JUSTICE

BETWEEN:	)	
	)	
DEBORAH SLAGHT	)	
	)	Corina Bachmann, for the Plaintiff
Plaintiff	)	
	)	
– AND –	)	
	)	
ROBERT B. PHILLIP and PETER	)	David Messam, for the Defendants
WICAARTZ	)	
	)	
Defendants	)	
	)	
	)	
	)	HEARD: May 10, 12, 13, 17-20, 2010 and October 18-22, 2010.

TURNBULL J.

JUDGMENT

- [1] The 47 year old plaintiff has brought this action for damages sustained by her in a motor vehicle accident on December 22, 2004. The defendant Robert Phillips was driving a motor vehicle (a passenger van) owned by his co-defendant Peter Wicaartz. The defendant denied liability and argued that the injuries of which the plaintiff was complaining at the time of trial were unrelated to the accident. The issue of causation was significant in this trial.
- [2] The plaintiff contended that in the motor vehicle accident, she injured her subscapularis tendon which progressed over time to a full tear, necessitating surgical repair approximately two years post accident. When the surgery was not successful in reducing her shoulder pain, the plaintiff gave up the two jobs she had been doing before the accident, and which she continued to do on a modified basis after the accident, up to shortly before the surgical repair of her shoulder in January 2007. She retrained to do a less physical job in a health care administration position but her income was reduced as a result. She sought damages for future loss of income, as well as general damages for pain, suffering and loss of enjoyment of life.

Facts of the Accident

- [3] The accident occurred on Provincial Highway 24. It is a two lane paved highway with gravel shoulders on each side of the road. On the day of the accident, it was snowing and the highway surface was snowy and slushy.
- [4] The plaintiff, who is 48 years of age, (age 42 at the time of the accident) testified that she was driving in the southbound lanes towards Simcoe from her job in Brantford at about 3:15pm. It was snowing at the time. There were cars proceeding southbound in front of her. She suddenly saw a car in front of her sway a bit and then she was confronted with the defendants' van coming across the centre line into her lane of traffic. The vehicles collided in the southbound lane and she estimated she was travelling at approximately 70 kilometers per hour just prior to impact. The front end of her vehicle collided with the side of the van.
- [5] Upon collision, she recalled that she was thrown forward and backward and then her body was thrown into the driver's door. She believed her hands were on the wheel at the time of impact but could not be certain because of the speed at which events transpired. Her 1994 Ford passenger vehicle was written off by her insurer after the accident but she was not certain to what extent the age of the vehicle governed that decision. After the accident, she remained in her vehicle and was taken to the hospital in an ambulance for examination.

Liability

- [6] The defendant Robert Phillips testified that he was driving northbound on Highway 24 at approximately 65 kilometers per hour. He was following four vehicles and estimated that he was at least 20 car lengths behind the vehicle in front of him when he began to lose control of his vehicle. He tried to bring it under control but ended up drifting across the centre line into the southbound lanes. He sideswiped one southbound vehicle and then crossed over to the west shoulder of the highway. As he did that, the right rear axle area of his vehicle collided with the front passenger side of the plaintiff's vehicle.
- [7] Mr. Phillips testified that when his car started to fishtail, he took his foot off the gas, tried to brake and put it in neutral to stop the van from sliding. During his cross examination he was confronted with his testimony at his examination for discovery where he said that he did not try to brake or at the very best, he was not sure of that fact.
- [8] I find that the defendant lost control of his motor vehicle because he was driving too fast for the road conditions. None of the other vehicles travelling northbound in front of him encountered any similar difficulties.

- [9] I found the evidence of Donna Tune, an independent witness, to be very helpful to the court. She was driving southbound in front of the plaintiff's vehicle. She stated that it was snowing heavily at the time of the accident and the road was slippery. She remembered that at one point on the highway, she was travelling about 65 kilometers per hour but she decided to slow down because she felt it was too icy. She was not sure of her speed at the time of impact. Just before the accident, she saw a northbound van fishtail towards her and then it appeared the driver got it under control. Suddenly it veered towards her vehicle again and narrowly missed her rear door. She looked in her rear mirror and saw it hit the plaintiff's vehicle. She came back to the accident scene and saw that the driver of the car (the plaintiff) was quite upset.
- [10] I find that the defendant failed to keep his vehicle under control and it was due to his negligent driving that the motor vehicle accident took place. His efforts to brake his vehicle would not have been necessary if he had been driving slower in such weather conditions.

Treatment Post Accident

- [11] The plaintiff was taken to the Simcoe General Hospital emergency department where she waited to be treated. That department was busy that afternoon and so she was not treated very promptly. After an hour or so, she testified that she was not able to sit comfortably there any longer so she left with a friend who gave her a ride home.
- [12] Ms. Slaght stated that when she got home she took some pain medication to relieve the discomfort. She did not sleep very well and went to see her family physician, Dr. Johnston, the very next day. She remembered complaining to him about her chest, her knee and her back. The defence has argued that this is significant because if she had suffered a complete or partial tear of the subscapularis tendon in the motor vehicle accident, she would have immediately noticed sharp pain in the anterior part of her shoulder and most likely swelling and bruising in the area would have been visible in the day or days following the accident. Ms. Slaght recalled that Dr. Johnston prescribed her some medications and arranged for her to take physiotherapy.
- [13] Dr. Johnston testified. He was an impressive witness. He has been a family physician for 44 years in Simcoe. He has been the plaintiff's doctor since 1986 and he described her as a "low maintenance" patient. He has found her to be compliant and essentially a healthy lady. He reviewed his clinical notes after the subject accident which indicated that he saw the plaintiff on December 23, 2004, the day after the motor vehicle accident. He noted that she complained of chest heaviness, painful knees, and discomfort in the neck and lumbar spine. He diagnosed her as suffering from myofascial injuries and prescribed ibuprofen and physiotherapy. He agreed in cross examination that he did not specifically examine her left shoulder during that visit to his office.
- [14] The plaintiff testified that she only took a few days off work after the accident. She explained that she was in the process of separating from her husband and that she could not afford not to work. Hence, she returned to her jobs at Atlas and Curves, doing modified work which is more fully explained below under the heading Wage Loss Claim.

- [15] The plaintiff started physiotherapy on December 31, 2004, under the care of Pauline Yurichuk at Simcoe physiotherapy. The Plaintiff recalled telling Ms. Yurichuk how the accident happened and the areas of her body in which she was feeling pain.
- [16] Pauline Yurichuk testified that she has been a physiotherapist for 43 years. She has practiced in Simcoe for approximately 40 years. She emphatically described the plaintiff as very compliant, honest and highly motivated whenever she had to deal with her. She further noted that Ms. Slaght had always been very athletic.
- [17] On December 31, 2004, the plaintiff stated to her that her whole body was hurting. The Plaintiff complained of constant burning in her knees, a tight ache in her neck and over the top of her left shoulder and the back of both shoulder blades. She recalled that Ms. Slaght had a lot of weakness when she tried to lift her arms above shoulder level. The plaintiff testified that she made the markings on the pain diagram found in Ms. Yurichuk's notes (exhibit 1, tab 13, page 37), specifically the markings showing pain on the top of her left shoulder.
- [18] Ms. Slaght recalled that the first week after the accident was very difficult for her. She had to go back to work and work part days during the holidays. When she commenced her physiotherapy sessions, Ms. Slaght remembered that she was forced to stretch her muscles, to do exercises focused on her knees, her chest and the tightness in her chest and received manipulation of the shoulders supplemented by heat and ultra sound treatments.
- [19] On January 20th, 2005, Ms. Yurichuk noted that the plaintiff's complaints mainly dealt with burning and sharp pains in her knees, especially the left. The plaintiff said she had just worked a thirteen hour day. She had trouble doing her leg exercises due to the discomfort in her knee.
- [20] At her next appointment on February 1, 2005, the plaintiff complained to Ms. Yurichuk of a very sore left shoulder. On examination, the posterior rotator cuff muscles were painful, especially when Ms. Slaght lifted above shoulder level at work. Two days later, she complained that her left shoulder was locking. On observation, Ms. Yurichuk noted that the left shoulder was forward. The plaintiff experienced discomfort and felt impingement in the shoulder when she tried to lift the arm.
- [21] Ms. Slaght stated that she then had a cortisone injection to her left shoulder on February 11, 2005. She testified that she had been having a lot of pain in her shoulder and when Dr. Johnston was not available, she went to a Walk in Clinic. She recalled that while the cortisone helped to reduce the pain in her shoulder, it did not take all the pain away but certainly made it more bearable. I find that the plaintiff clearly was symptomatic in the left shoulder to go to such lengths as to go to a walk in clinic to take such an injection. In my view, based on the medical information before this court, the only event that could have triggered such a painful reaction was the motor vehicle accident of December 22, 2004. The temporal relationship is, in my view, not simply a co-incidence.

- [22] When Ms. Yurichuk saw the plaintiff on February 14, 2005, she recorded that Ms. Slaght had experienced increased pain in her left shoulder when she was trying to demonstrate an exercise at Curves. Because of her discomfort, she explained to Ms. Yurichuk that she had gone to the Walk in Medical Clinic and had been given a cortisone shot into her left shoulder which had reduced her pain.
- [23] At each of the three other attendances in February, Ms. Slaght complained of continuing discomfort in her left shoulder which was aggravated by particular actions. On March 8, 2005, Ms. Yurichuk reported to the plaintiff's accident benefit insurer that the plaintiff was able to work and do household tasks and was functioning at about 80% of her pre-accident level. She noted, however, that the plaintiff who had been excellent in her compliance and attendance at physiotherapy, might require further treatment for her knee and left shoulder.
- [24] Ms. Slaght testified that after the cortisone shot reduced her pain, she was discharged from physiotherapy and the pain remained at a reduced level for several months. She continued doing the exercises at home which Ms. Yurichuk had taught her. As the effects of the cortisone shot receded, she began taking more and more pain medication and finally went back to see her physiotherapist on December 9, 2005 to get help for her discomfort.
- [25] Ms. Yurichuk noted that the plaintiff complained that her left shoulder had not healed since her motor vehicle accident. The plaintiff advised her that she had continued to work at her two jobs but had been unable to work out herself due to continuing problems with her left shoulder and her left knee. She also complained of discomfort in her left wrist for the preceeding three months due to having to compensate for the pain in her left shoulder. The pain diagram prepared by Ms. Yurichuk at the time of that attendance (exhibit 1, part 1, tab 14, page 55) clearly shows that the left trapezius, shoulder and left wrist are identified as areas of discomfort. On the same page, she has noted that with external rotation of the left arm, the plaintiff experienced pain through the rotation of movement and she exhibited weakened resistance. The same findings were noted with internal rotation and abduction of the arm. At page 65, the plaintiff had noted that she was experiencing severe shoulder pain when she was performing any specific activity and that she had been having severe difficulty sleeping because of pain in her arm, shoulder or hand.
- [26] Of significance to this court is the fact that the plaintiff, who was an avid recreational golfer, had to give up some of her golf during the summer of 2005 due to the grinding in her shoulder. I find that there was a progression of symptoms in her left shoulder during 2005 and that they had become markedly more severe by the time of her attendance with Ms. Yurichuk in December 2005.
- [27] On February 15, 2006, Ms. Yurichuk noted that the plaintiff's discomfort in her neck and shoulders had reduced by almost 50%, but that the plaintiff had put on weight due to her physical inactivity. When the plaintiff attended at her office on March 8, 2006, she complained of significantly increased pain in her left shoulder which had been caused

when she moved her left arm out and to the back. Ms. Yurichuk testified that each time she had seen the plaintiff, it seemed that she had decreased functional use of the left shoulder and increased clicking in the shoulder. She recalled that about that time, she began to suspect that she had a significant shoulder injury and should be seen by an orthopaedic specialist.

- [28] Dr. Johnston did see her with respect to unrelated matters during 2005 and the plaintiff did not mention any problems with her left shoulder until her attendance with him on March 21, 2006. He explained that from December 2004, she was treated by her physiotherapist and so he did not see her much in the interim because physiotherapy was the recommended treatment. He agreed in cross examination that it was surprising to him that she had not mentioned any problem with her left shoulder before March 2006. On March 21, 2006 she complained of persistent pain in her left shoulder with increased pain on abduction of the left arm. He decided to order an X-ray and ultra sound. When he got the results of the ultra sound around June 9, 2006 they appeared to show a partial tear of the supraspinatous muscle. He referred her to be seen by a surgeon.
- [29] When Ms. Yurichuk requested the plaintiff's accident benefit insurer to fund 12 more treatments for her left shoulder, the insurer refused. On May 31, 2006, the plaintiff had an insurer's independant medical examination and following that, Ms. Yurichuk was advised that the insurer would fund 12 more treatments. At the same time, the plaintiff was given approval to purchase a TENS machine so she could give herself treatment at home. Ms. Yurichuk noted that the plaintiff was still working but was very frustrated about her pain in her left shoulder.
- [30] By late June 2006, the plaintiff advised Ms. Yurichuk that she was cautiously trying to play some golf and her notes indicate she was using a modified swing. Ms. Slaght testified that she believed she had tried to play golf in the summer of 2006 but her pain in her shoulder increased to a point where it was no longer enjoyable. After her surgery, she has tried to play golf a few times by just using an easy swing, but has since given it up due to the shoulder discomfort occasioned by playing.
- [31] On August 16, 2006, the plaintiff advised her physiotherapist that she had had another cortisone shot about two weeks previously which had decreased her pain. The plaintiff identified her pain as as a low grade ache, restricted to the subscapularis and upper trapezius area. She stated that she had to be careful with her movements but exhibited a full rotation of movement of the left shoulder but experienced pain on internal rotation. She explained to Ms. Yurichuk that she wanted to save her other insurance funded physiotherapy treatments she needed them more, because her shoulder had settled down after cortisone shot of July 31, 2006.
- [32] Ms. Slaght testified that she decided to return to the Walk in Clinic to get the second cortisone shot on July 31, 2006 to reduce the discomfort she was experiencing in her left shoulder. She felt it helped but only for a month or so.

- [33] Finally, she met with the surgeon to whom she had been referred by Dr. Johnston. Dr. Rajaratnam discussed the possibility of surgery with the plaintiff on August 30th. He had determined that she was mainly exhibiting symptoms of impingement which seemed to be controlled by cortisone. He recommended an impingement release surgery which he told her would improve her condition but not make her perfect. The plaintiff elected to have the surgery and it occurred January 12, 2007.
- [34] Shortly after her surgery, Ms. Slaght began gentle physiotherapy treatments involving the manipulation and rotation of the shoulder. She recalled that her pain was somewhat reduced, but not totally eliminated. She had to use her right hand (her non-dominant hand) for everything as her left arm was of no use to her. She recalled taking a lot of pain medication. Her day to day functioning around the house was difficult as she could not do housework. She was not able to return to work. As time passed, she found that she was not able to use the left arm and her pain was not getting any better. She received Income Replacement Benefits from her automobile insurer. A note in Ms. Yurichuk's clinical notes dated April 23, 2007 records the plaintiff's frustration with her lack of functional progress and continuing pain at a 3/10 level. She was given a further cortisone shot in her left shoulder on April 30, 2007.
- [35] The plaintiff testified that after a few months, she was disappointed that the pain in her shoulder was not significantly reduced. As a result, she decided to seek legal counsel and this action was instituted.
- [36] In an effort to assist her patient, Pauline Yurichuk urged the plaintiff to seek treatment from another physiotherapist, who also worked in Simcoe Physiotherapy. She took Ms. Slaght to a gym and taught her a regime to maintain her upper body strength. She went to that gym two to three times per week as part of that program. Since then, she has stopped going to the gym as she has a treadmill and weights at home where she can do the needed exercises.
- [37] In the spring of 2008, she was referred to Dr. Jacques Gouws who is a psychologist. She explained that she was very down in her mood due to her lack of medical progress and her inability to do many of the things she had previously done, including her work at Atlas and Curves. She explained that the treatment with Dr. Gouws and his staff taught her strategies to turn things to the positive from the negative.
- [38] In the autumn of 2008, Ms. Slaght testified that she was referred to Dr. Dinesh Khumbare, a specialist in rehabilitation medicine. He suggested a treatment plan of botox injections to her shoulder, her neck and between her shoulder blades to try to reduce her pain. She explained that when she gets an injection, there are 15-20 injections into the muscle of her left shoulder which reduces the pain. She has continued this course of botox treatment every two to four months up to the time of trial. She returns when the pain increases.

- [39] In the summer of 2008, she also began a course of massage treatments to relax the muscles.

At present, Dr. Johnston has prescribed Dorvan and Tylenol 3 for pain.

Pre Accident Medical Condition of the Plaintiff

- [40] Ms. Slaght had seen Ms. Yurichuk on and off from 1999 to March 2001.
- [41] Ms. Yurichuk referred to her notes (Exhibit 1, Vol 1, tab 9) as she testified. The first time that she had ever seen the plaintiff was in April 1999 after she had been involved in a motor vehicle accident. The plaintiff then complained of headaches and tightness of the shoulder muscles. The diagram at page 2 showed the pain distribution down the back of the neck to the upper back. Of interest, her notes indicated that on May 10th, 1999, the plaintiff was complaining of increased pain, but had been able to play baseball the night before. On June 4, 1999, she noted that the plaintiff had fallen on her right shoulder playing baseball. The other entries in her notes to the time of initial discharge in September 1999 indicate that while the plaintiff was experiencing some discomfort in her right shoulder and some crepitus in her neck, she was able to continue her baseball and going to the gym. Her discharge report at page 16 indicated that the plaintiff was able to all activities of daily living, work overtime, play baseball, return to the gym and to do Tai Bo. She stated that the plaintiff had been a very compliant patient.
- [42] When the plaintiff returned to her care in January 2000, another pain diagram was done (page 20) demonstrating pain at the base of the neck radiating down the spine into the thoracic area between the shoulder blades. In a report to the plaintiff's insurer dated March 14, 2000, Ms. Yurichuk noted that the plaintiff was limited with heavy lifting at work and had demonstrated excellent compliance. She suffered a further flare up of symptoms which ultimately led Ms. Yurichuk to write a letter to the insurance adjuster stating that the plaintiff had been working on modified duties and had not been able to resume her regular job which required lifting. In that letter, she commented that certain actions were aggravating her residual neck and shoulder pain. Ms. Yurichuk expressed the opinion that the plaintiff would not be able to return to her regular duties as a factory assembly worker.
- [43] I observe that despite that letter, the plaintiff did continue her work as a factory worker. Of import, there was no complaint at all of injury to the left shoulder in all these notes.
- [44] Ms. Yurichuk discharged the plaintiff from her care on March 1, 2001. She urged her to return to the clinic for further care if her pain and stiffness did not subside. She did not see her again until December 31, 2004, nine days after the motor vehicle accident which is the subject of this action.
- [45] Dr. Johnston, her family doctor, indicated that there were no pre-accident complaints from the plaintiff relating to any problems with her left shoulder or her left arm.

Post Accident/Pre Surgery Accidents of the Plaintiff

- [46] The plaintiff was involved in a number of incidents in the months between the December 22, 2004 motor vehicle accident and her surgery of January 2007. She denied that any of them injured or aggravated her already injured left shoulder. The doctors who testified before this court found no indication in any of the medical records that there had been any complaints recorded of injury to that part of her body and so they agreed that those incidents did not cause or materially contribute to the plaintiff's left shoulder problem. I concur.
- [47] In April 2005, the plaintiff and a friend were riding on a motorcycle. While going slowly to effect a right turn, their bike was hit by another slow moving motorcycle driver in front of them. The plaintiff testified that she fell to her right and she only suffered a cut on her left leg. She testified that her left shoulder was not injured in the accident and she did not seek any treatment for her left shoulder. I accept her evidence on this issue.
- [48] On March 8, 2006, the plaintiff complained to her doctors that she had reached behind her and externally rotated her arm and felt a sudden, sharp pain in her left shoulder. There is no suggestion in the evidence or the medical records that there was any trauma or other intervening force which would have caused such pain when making a movement that would ordinarily cause no discomfort to a person with a healthy shoulder. I find that this incident is consistent with the progressive deterioration of the plaintiff's left shoulder due to the presence of a partial subscapularis tendon tear.
- [49] On September 18, 2006, the plaintiff reported to Dr. Johnston that she had fallen down the stairs in her residence. On examination, he found she had suffered multiple contusions with pain over her right rib cage and a swollen right middle finger. He ordered x-rays which were negative. There was no further medical follow up relative to the injuries sustained in this incident. He testified that the plaintiff later admitted to him that she had been assaulted and that she had fallen down the stairs. Ms. Slaght testified that she initially told Dr. Johnston that she had fallen down the stairs because of her embarrassment about this incident which she considered to be a very personal part of her life. I accept her evidence and find it understandable that she would have tried to avoid giving all the details to her doctor at the time. I accept her evidence that she did not suffer any injury to her left shoulder at this time.

Causation:

- [50] The court heard from three witnesses on the relationship of the complete tear of the plaintiff's subscapularis tendon to the motor vehicle accident.

Evidence of Dr. Rajaratnam

- [51] The plaintiff called Dr. Krisham Rajaratnam to testify. He is the surgeon who operated on the plaintiff to repair her fully detached subscapularis tendon in January 2007.
- [52] Dr. Rajaratnam is a specialist in surgery to the upper limbs. He is a staff surgeon at the Hamilton Health Sciences Corporation and does all the shoulder replacements and

arthroscopic upper extremity procedures at that center. He noted that he has only done six medical legal assessments in the past six years.

- [53] He stated that a tear in the tendons of the rotator cuff can start as partial thickness tear and proceed to a full thickness tear. He said the process is analogous to the process by which a sock exhibits a full hole after having worn thinner and thinner by usage over time. He described a partial thickness tear is when one can see light through the thinning of the tendon but a hole in the tendon is a full thickness tear.
- [54] He stated that the rotator cuff tendons work together as a balanced force to permit the patient to elevate the arm above shoulder level. If any of the tendons are worn, torn or inflamed, the arm can not be easily lifted above the shoulder level.
- [55] Dr. Rajaratnam first met with and examined the plaintiff on August 30th, 2006 on referral from her family doctor. He found no obvious wasting or muscle bulk loss in the left shoulder. He explained the significance of that finding was that if the tendon was torn, the muscle does not work and it loses bulk. He found that the plaintiff had a full range of motion of the shoulder but remarked that a full range of motion does not tell the examiner anything about pathology. He noted that the plaintiff had had cortisone injections into the shoulder which might have relieved her pain to allow for an easier range of pain free motion. He remarked that if a person had a complete tear of the subscapularis tendon, she would not be able to do the “lift off test”. However, if the tendon was only partially torn and the pain had been removed by something such as a cortisone injection, the damaged tendon would allow her to lift her hand off as part of the “lift off test”. He stated that the “lift off test” is not sensitive enough to identify a subscapularis tear because if the test is normal, the problem can still exist.
- [56] As part of his clinical assessment, he looked at an ultra sound report dated April, 2006. On the ultra sound reading, there was evidence of an intra substance tear but no evidence of a subscapularis tendon tear. He stated partial thickness tears are very difficult to see on an ultra sound reading and much depends on who did the ultra sound and the quality of the ultra sound itself. He noted that subscapularis tears are rare, making up only about 30% of rotator cuff tears.
- [57] Dr. Rajaratnam stated that he was surprised to find a full subscapularis tendon tear when he operated on January 12, 2007. He estimated that in about 5% to 10% of his surgical procedures, he is surprised at the findings during surgery. He found the tendon was torn where it inserted into the lesser tuberosity on the humeral head. To repair it, he had to release the scar tissue stopping the tendon from coming back to that insertion point. He noted that the tendon quality was good enough to hold the tissue as he only had to use one anchor. He described the anchor as a wall plug into the bone and the tendon goes around the plug. In his view, it was probable that the tear was more than two years old.

Evidence of Dr. Robert Martin

- [58] Dr. Robert Martin, an orthopaedic surgeon, was retained by the plaintiff to review the file and to offer an opinion on the relationship of the plaintiff’s left shoulder injury to the

motor vehicle accident. He has continued to do diagnoses, assessments and treatment in a medical clinic in Hamilton since his retirement from surgery in 1997. He regularly sees patients referred to him by other physicians dealing with a variety of musculoskeletal problems. He estimated that he does approximately eight consultations per week and about thirty per month. Of those patients he sees, approximately 80% complain of chronic pain, often related to arthritic knees, shoulders and back conditions. He also does medical legal assessments as part of his present practice and estimates that he does approximately 120 of these per year. Most of those deal with motor vehicle accidents and a majority have chronic pain implications. He estimated that he does two assessments for defendants to every one he does for a plaintiff.

- [59] He recalled meeting the plaintiff for the first time in 2008. He had received the relevant medical information for the plaintiff before he saw her. He interviewed her and obtained a full history from her. After conducting a physical examination, he then made a diagnosis of her impairments, their relationship to the motor vehicle accident and made a prognosis for her future recovery. He stated that he generated his report within 24 hours of having seen the plaintiff.
- [60] As a result of his assessment, he formed the opinion that in the motor vehicle accident, the plaintiff had suffered a tear of the subscapularis tendon of the left shoulder, subacromial bursitis of the left shoulder, degenerative arthritis of the left shoulder and that she suffered chronic pain of the left shoulder. He stated that the first three conditions were found by Dr. Rajaratnam during his surgical procedure. He noted that Dr. Rajaratnam's description in his operating note in which he described chronic thickening of the subacromial bursa and marginal spur formation of the acromium as clear evidence of arthritis in the region of the subacromial joint. He felt that the degenerative arthritis in the shoulder joint was most probably caused by the injury sustained to that joint. He remarked that the rest of his musculoskeletal examination of the plaintiff was normal.
- [61] He found that her major impairment was pain and limitation of the left shoulder. She was not able to elevate her arm above shoulder level and thus was limited in certain work that she could do. He felt, in the absence of any other cause, her limitations were due to the motor vehicle accident. He felt that she was at a plateau of recovery at that time.
- [62] Dr. Martin also stated that he has found that pain emanating from the shoulder area is often diffuse and is often not specific. It is frequently accompanied by pain from the neck and it is sometimes difficult to dissociate one from the other. In his view, the pain complained of by the plaintiff shortly after the accident was emanating from the shoulder. Because there was no prior evidence of injury to the left shoulder, and because of the continuity of left shoulder pain after the motor vehicle accident, he was of the opinion that the plaintiff had an incomplete tear of the subscapularis tendon initiated in the motor vehicle accident of December, 2004 which subsequently progressed to a full thickness tear.

Evidence of Dr. Erin Boynton

- [63] The defence called Dr. Erin Boynton, who is an orthopaedic surgeon, to give expert evidence with respect to the relationship of the tear of the subscapularis tendon to the motor vehicle accident. She is a very well qualified expert with significant expertise in shoulder surgery and surgery related to the subscapularis tendon. She has not been performing surgery as part of her practice since 2006 but has been concentrating her practice since then on research and providing medical/legal opinions to insurers and counsel. She recently participated in the publication of an article dealing with the definition of the subscapularis tendon and the digitization of every tendon of the subscapularis tendon. She also recently authored an article in the *Journal of Bone and Joint Surgery* dealing with conservative treatment of injury to the rotator cuff.
- [64] She prepared two reports, one dated March 10th, 2010 and the second dated in May 4, 2010 to reply to the “reply” report of Dr. Martin, which were served pursuant to Rule 53.03(1) of the *Rules of Civil Procedure*. In preparing her two reports, she reviewed the medical brief provided to her, including the reports of Dr. Martin. She did not examine or interview the plaintiff, but I do not think that in light of the fact she was retained approximately two years after the plaintiff had her surgery, that factor affected her ability to offer her opinion on the issue of causation.
- [65] She testified that tears of the subscapularis tendon are quite unusual and there are two ways in which such an injury most often happens. The most common is caused by degeneration of the tendon as the collagen wears down. She agreed that such a tear would be a rare event in a person of forty years of age or less. She further agreed in cross examination that she had not often seen people in their early 40’s with a degenerative tear in their subscapularis tendon but added that it can occur.
- [66] Dr. Boynton attributed trauma to the shoulder as the second most common cause of such injury and to tear the tendon, a significant stretch or tension would have to be applied to the tendon because it has a depth of at least one centimetre. She suggested that such trauma would require the arm being abducted from the body and externally rotated so that the hand rotates behind the head. This extreme motion would have to be associated with violent trauma and a dislocation of the shoulder. In medical terms, she described this as displacement of the humeral head from the glenoid cavity which would allow extra tension to be applied to the tendon.
- [67] Dr. Boynton agreed that none of the incidents involving the plaintiff prior to the incident motor vehicle accident identified an injury to the left shoulder. Thus, she submitted that the court was left with the plaintiff’s assertion that her problem began with the accident of December, 2004. Dr. Boynton found such a suggestion was not tenable for four clearly articulated reasons.
- [68] First, if there was a traumatic tear, she felt that one would expect immediate pain localized to the front of the shoulder where the subscapularis tendon inserts on the upper part of the humerus. Most patients experiencing such an injury immediately feel pain in the upper shoulder radiating into the upper arm. Her review of the medical records suggested that the plaintiff had suffered soft tissue injury to the neck and shoulder due to

the nature of her complaints and the distribution of the pain at the back of her neck along the trapazius along the neck. She did not feel that the shoulder was injured.

- [69] Secondly, there would be bleeding associated with such a tear which would lead to bruising being visible in the affected area. On physical examination, one would expect to see swelling, limited range of motion, bruising and significant pain. She noted that neither Dr. Johnston or Pauline Yurichuk commented on such findings in their clinical notes in the nine days immediately following the accident.
- [70] Third, if a tear to the subscapularis tendon was suspected, an attending physician would do specific tests to try to detect which shoulder muscle was damaged by stretching the shoulder and determining what loss of function was exhibited. Tenderness on external rotation of the arm and inability to perform a “lift off test” would both be indicators of injury to the subscapularis tendon and/or muscle. From her review of the file, she did not see that any of these tests were performed. However, the clinical notes indicate that the plaintiff had full rotation of movement after the accident.
- [71] Fourth, if the motor vehicle accident had led to a complete tear of the subscapularis tendon it is not common to have the surgery to repair it almost two years after the traumatic event because it is very difficult after such a passage of time to reverse the process of the tendon pulling away from the bone from which it has been torn. Dr. Boynton stated it would be almost impossible to repair after two years. She felt that the tear of the tendon may have been much more recent than December 2004 because the surgeon who performed the repair in January 2007 was able to remobilize the tendon and bring it back to the bone. She noted that there is a difference in the reparability of a degenerative tear and a traumatic tear.
- [72] Dr. Boynton agreed that with a partial thickness tear, there may be less swelling and less bruising evident. However, she added that there is often more pain with a partial tear than with a full thickness tear because the nerve fibres in the structure become hypersensitive to the inflammation. While one would expect to find a limited degree of motion, the reduction of external rotation of the arm would usually not be as pronounced as with a full thickness tear because the tendon is still attached to the humerus. She felt that initially, after a partial tear, the patient would have a great deal of difficulty doing anything due to the pain and swelling. But then after a period of time, she stated the patient can often function quite adequately if the other three tendons of the shoulder adapt to compensate for the reduced function of the subscapularis tendon.
- [73] In cross-examination, she agreed that she never expressed an opinion in her reports that the cause of the tear was due to degenerative changes. She explained that she focused most of her opinion on why she thought that the injury could not have been traumatic in origin. She stated that the tear began at some stage but it was difficult to identify specifically when that commenced.

Analysis of Causation Issue

- [74] I accept the evidence of Dr. Rajaratnam and Dr. Martin that the subscapularis tendon was probably torn in the motor vehicle accident and that it progressed to a full thickness tear necessitating surgery. I find it is the direct result of the motor vehicle accident and that the plaintiff's present limitations with her left arm and shoulder and the pain and effects upon her life are directly due to the injury sustained in the motor vehicle accident of December 22, 2004.
- [75] Dr. Boynton stated that it would have been almost impossible to surgically repair the subscapularis tendon after two years if the motor vehicle accident had caused a full thickness tear. I find that the fact that the repair was able to be effected and the process of the tendon pulling away from the humerus reversed, supports the finding that the full thickness tear was progressive and did not occur until a much later time. Dr. Martin felt that the full thickness tear would have caused her severe pain when it occurred. I note that on March 8, 2006, the plaintiff complained of severe pain in the left shoulder when she reached behind her and externally rotated her shoulder. This is the very motion Dr. Boynton described as the most common cause of traumatic injury to the subscapularis tendon but she indicated it would have to be significant trauma. She did not comment on whether a partially torn subscapularis tendon could be advanced to a full tear by such a motion.
- [76] Dr. Boynton argued that the absence of localized anterior pain in the left shoulder immediately after the accident supported her opinion that the trauma of the motor vehicle accident did not cause a full or partial tear of the subscapularis tendon. Dr. Rajaratnam was firm in his rejection of that opinion. He stated that shoulder pain can be diffuse in nature. If it is attributed to a specific injury to a tendon, it often is not well localized. He said that unlike a finger, which is well enervated, pain in a shoulder can be felt anywhere from the neck to the shoulder at first and with the passage of time, the identity of the pain becomes more localized. Hence, he found the complaint of neck and trapezius discomfort by the plaintiff to Ms. Yurichuk not to be inconsistent with a possible tendon tear because the shoulder pain can initially be very diffuse and it can extend anywhere from the shoulder girdle to the neck or elbow. I accept the evidence of Dr. Rajaratnam on this issue.
- [77] Dr. Rajaratnam further found that the fact that she worked for almost two years after the accident in a rather physically demanding job could easily make a partial thickness tear progress to a full thickness tear.
- [78] Dr. Rajaratnam also felt that a person gripping the steering wheel just before a head on collision could have a tear of the subscapularis tendon or muscle caused by a sudden acceleration/deceleration which could transfer force to damage the tendon. He doubted that she could have worked for two years post accident with a full thickness tear but he stated that it was possible she would have been able to do so with a partial tear if the cortisone gave her pain relief. He stated that the cortisone can provide relief from 6 weeks to forever for different patients. He noted that she had had a cortisone injection not long before he first saw her in August, 2006. The records indicate that was done on July 31, 2006. (exhibit 1, part 111, tab 7, last page). When he examined her in August

2006, she had a full range of motion of the left shoulder which he stated would be consistent with a partial tear and a reduction of pain from the cortisone injection. He agreed, that a traumatic event could have occurred between the time he saw her in August and the time of the surgery which caused the complete tear. However, he further agreed that a partial tear can progress to a full tear, by normal wear and tear and a traumatic event is not required to make that happen.

- [79] The pain diagram done by the plaintiff in the office of Pauline Yurichuk at her first post accident physiotherapy session clearly indicates pain in the area of her left shoulder area. This contrasts clearly with earlier pain diagrams in her clinical notes from treatment sessions from 1999 to 2001.
- [80] On January 11, 2005, the plaintiff complained of weakness in her upper extremity to her physiotherapist. On February 1, 2005, Ms. Yurichuk noted that the plaintiff was complaining of pain in her left shoulder. On February 3, 2005, Ms. Yurichuk noted a complaint of left anterior shoulder pain, which is evidence Dr. Boynton described as indicative of trauma to the subscapularis tendon. On February 11, 2005, the plaintiff visited the West Street Health Walk In Clinic complaining of left rotator cuff pain burning up into her neck and shoulder. At that time, the treating physician gave her a 40 milligram injection of Kenalog (a type of cortisone).
- [81] Ms. Slaght explained that she did not disclose her ongoing shoulder problem to Dr. Johnston during 2005 because she was being treated by Pauline Yurichuk for that problem two or three times per week until she had her first cortisone shot which helped settle down her symptoms. I accept her explanation as being truthful and logical.
- [82] I also find that she was very motivated to get back to her normal lifestyle. Her withdrawal from her friends and her reaction to her limitations after the surgery are not surprising for a person who previously was so athletic. She took reasonable and responsible steps to try to return to Atlas, to seek further education, and to then locate a job in a timely manner.
- [83] Dr. Johnston and Pauline Yurichuk both stated that she was a person who was an excellent patient and always tried to comply with recommended treatment. She was not considered to be a difficult patient. She complained about her left shoulder pain by February 1, 2005. Dr. Rajaratnam testified that often the pain in the shoulder is diffuse and difficult to localize at the outset after an incident. On the record before this court, it is clear that the plaintiff was complaining of significant pain in her left shoulder by early February 2005.
- [84] Furthermore, at no time did Dr. Boynton suggest in her reports that the tear of the subscapularis tendon was degenerative in nature. I would have expected that a doctor, who does many medical legal assessments each week, would have expressed an opinion that the tear was degenerative in nature, if that was her view. Dr. Boynton clearly said that degeneration of the subscapularis tendon or muscle is not common in people in their early 40's. That comment, when combined with the complaints of the plaintiff with

respect to her left shoulder within 6 weeks of the motor vehicle accident, satisfy me on the balance of probabilities that the motor vehicle accident was the cause of the partial tear in the plaintiff's subscapularis tendon which progressed over the next two years to a full thickness tear. The progressive nature of her complaints and efforts to treat the increasing pain confirm that finding.

- [85] Dr. Boynton felt that the mechanism of the injury was not consistent with the injury. However, she focused on a complete tear of the subscapularis tendon at the time of the accident and in my view, did not consider the possibility that a partial tear had occurred which ultimately progressed to a full thickness tear. She agreed that she based her opinion on an expectation of what would have happened to the plaintiff's arm at the time of the accident. She further agreed that she did not have the complete information at the time of her consideration of this case including what did happen to the plaintiff's arm at the time of impact and the presence or absence of air bag deployment.
- [86] During her cross examination, Dr Boynton was confronted with her March 3rd, 2010 report in which she stated that when the plaintiff saw Dr. Rajaratnam in August 2006, her fall down the stairs could have caused the subscapularis tear. This is not what she testified was a probable cause during this trial. She was also confronted with her theory expressed later in the same report that the motorcycle accident in which the plaintiff was involved could explain the subscapularis tear. This also was not what she testified was a probable cause during this trial. Again, she agreed during her testimony at trial that the absence of any complaints related to the left shoulder after those incidents made it highly unlikely the subscapularis tendon injury was caused by either one of them.
- [87] During her testimony at this trial, Dr. Boynton stated that she had not made mention of the March 8, 2006 incident because she only became aware of it when she received Dr. Martin's report dated March 2010. However, in cross examination she acknowledged that she did have the report of Pauline Yurichuk dated March 15, 2006 (exhibit 1, volumue 1, tab 14, page 92) which specified an incident consistent with a subscapularis tendon injury or aggravation. In her testimony, Dr. Boynton said the motion described in that report could have caused the partial tear or aggravated an existing tear or progressed an abnormality.
- [88] I found the evidence of Dr. Rajaratnam to be compelling. He was not hired by either of the parties. He was a treating surgeon who expressed his view that the motor vehicle accident was probably the initiating cause of the subscapularis tendon ultimately proceeding to a full thickness tear. Dr. Martin ultimately concurred in that opinion.
- [89] I find that the plaintiff has proved on a balance of probabilities that her left shoulder was injured in the motor vehicle accident. As I listened to the plaintiff testify, I found her to be fair and understated. She related her left shoulder problems to the motor vehicle accident. Absent any symptoms pre accident and absent any other intervening event between December 22, 2004 and the onset of acute, specific symptoms on or about February, 1, 2005, I find the plaintiff did suffer the initial subscapularis tendon injury in

the motor vehicle accident of December 22, 2004 which then progressed with the passage of time to a full thickness tear.

- [90] I find on a balance of probabilities that but for the accident of December 22, 2004, the plaintiff would not have suffered a partial or full thickness tear of her left subscapularis tendon which ultimately required surgical intervention. *Athey v. Leonati*, [1996] 3 S.C.R. 458; *Cottrell v. Gerrard et al*, 2003 CanLII 50091 (ON C.A.) I find that as a direct result of that motor vehicle accident which was caused by the negligence of the defendant Robert Phillips, the plaintiff has suffered and will suffer significant pain, suffering and loss of enjoyment of life and has had to change vocations which will cause her a future loss of income.

Wage Loss Claim

- [91] The plaintiff has sought damages for her short term wage losses and her long term wage loss as well as the loss of pension benefits.

Work History of the Plaintiff

- [92] The plaintiff began working at Atlas Hydraulics in Brantford in 1999. It is a factory that produces hydraulic hoses of varying sizes for various manufacturers. She started as a pusher who is expected to push both pieces of a hose together. She was working a 40 hour work week and, after a period of time, was promoted to a press job. Ultimately, she was promoted to a job involving capping, which is part of the inspection process to check fittings. At the time of the accident in 2004, she was a "floater" moving from job to job in the factory as needed and was paid \$14.50 per hour. She also received certain benefits including drug, dental, eye care, semi private hospital room coverage and three weeks of paid vacation. Her employer matched her contributions to her pension plan to a limit of 2 per cent of salary per year. At the time of trial, Atlas was still in business as a non-union shop. Gregory Perrott, who is the hose plant supervisor at Atlas, testified that he expects Atlas to be in business indefinitely. He noted that Atlas has three different plants in Brantford. He was always the plaintiff's direct supervisor from the time he became a supervisor in 2003 and said that she was the type of employee the company would like to have been able to continue to employ.
- [93] Ms. Slaght described the physical requirements of her job as medium to heavy, depending on the size of the hose, the size of the fitting and the nature of the packing that had to be done.
- [94] She also held a second job at Curves where she was paid \$8.50 per hour. It is a gym designed for any age of woman to work out. She started working in the Simcoe Curves facility in the late spring of 2004. In her earlier days, the plaintiff was a qualified fitness instructor. At Curves, she worked about 20 to 25 hours biweekly and would work 2 to 3 days per week on average. She testified that she enjoyed the job as it also allowed her to work out in the gym. She described the physical component of the job as being "medium" in nature because the machines were hydraulic. Her duties included showing people the machinery and making sure that they set each piece of equipment up properly and used it safely. She further consulted with clients with respect to diet plans. She had

telephone answering duties and at the end of the day, she was responsible to clean the facility so that it would be ready to be used in the morning.

- [95] After the accident, the plaintiff stated that she took a few days off work and then tried to return to work right away. At about that time, she was separating from her husband and she could not financially afford not to work.
- [96] When she returned to work at Atlas, her employer put her on modified duties so she was able to continue to work full time at 40 hours per week. She described the physical demands of her modified work as heavy but her supervisor allowed her to do the lighter work. She recalled that her supervisor Greg also trying to help her in any way he could. Thus, she was able to do her job, but not without pain and medication. By the spring of 2005, she recalled that she was doing sign offs for her supervisor as part of her modified work. It was not a strenuous job and simply required an observation of the customer's order to make sure that everything done matched that order. After December 2004, she was never able to return to pushing larger hoses like she was doing before her accident. She continued doing some crimping and sign off work during 2005 and up to the time of her surgery in January 2007.
- [97] The plaintiff testified that when she returned to work at Atlas she felt pain in her neck and in her left shoulder area. In due course the pain extended to her left elbow, and ultimately into her left wrist so that her whole upper left limb was affected.
- [98] After the accident, she also went back to her job at Curves because she needed the money. She recalled that she had a hard time going back to that job because she could not easily use the equipment after the accident. She was able to still answer the phone, do the contracts with the clients and demonstrate on some equipment. She asked some of the ladies who had been longer-term customers at Curves to demonstrate on the machines for new customers while she verbally described proper use of the machine. However, on Sundays, when there was no other member present, she would have to try to demonstrate the use of the machines which she found difficult. Finally, in October 2006, she was forced to stop working at Curves because she felt physically and mentally exhausted with the pain.
- [99] After her surgery in January 2007, she stopped working at Atlas because her shoulder pain never went away. When she realized that her shoulder was not improving several months after her surgery, she began working with a vocational counselor named Melissa Malacaria.
- [100] At or about the same time, she retained counsel with respect to this lawsuit and the Notice of Action to commence this litigation was issued May 14th, 2007. I find this of some significance as the plaintiff clearly anticipated that whatever injuries she had sustained in the motor vehicle accident would ultimately heal, until the surgery did not attain the expected results.

- [101] I note that in paragraph 24 of the Statement of Defence, the defendants pleaded that the plaintiff had failed to institute this action within the two year limitation period prescribed by the *Limitations Act, 2002*, S.O. 2002, c. 24. At trial, counsel for the defendant quite properly did not argue this as a defence to the action. In my view, the plaintiff clearly did not discover the severity of her injury until after her surgery. Even her surgeon did not realize there was a subscapularis tendon tear. After the surgery, the plaintiff understandably was hopeful that she would not be left with a permanent impairment in the use of her left arm and shoulder. I find on the evidence that the plaintiff instituted this action almost immediately upon discovering her injury was serious and may be of a permanent nature.
- [102] Ms. Malacaria's services were funded through the plaintiff's accident benefits coverage from late in 2007 through 2008. Ms. Slaght recalled discussing her vocational options with her family doctor, Dr. Johnston, who felt that retraining would be a good idea.
- [103] Ms. Malacaria discussed the possibility of the plaintiff returning to school to upgrade her training. Ultimately, she found courses suitable for the plaintiff at Fanshawe College, which would allow the plaintiff to obtain a certificate as a health office administrator.
- [104] Ms. Slaght did try to work at Atlas in the autumn of 2008 so that she could determine for herself if she was able to return to that job. She testified that for her it was important to try to do the job one more time before making the final decision to return to school. She recalled that upon her return to work, her employer ordered that she work at the lightest job in the factory. She started at two hours per day, twice per week to ease back into the workplace. She recalled that she had to increase her medication when she did work, and she worked to the end of December. After approximately eight weeks, the increase in her pain was significant. At that time, in conjunction with Ms. Malacaria, she decided it would make sense to go to school as she knew she could not return to Atlas on a full-time basis. The management at Atlas had asked her to increase her hours per week and Dr. Johnston and Ms. Malacaria did not think that was sensible.
- [105] In January 2009 she enrolled at Fanshawe College in a 16 week full time course. That was followed by a co-op placement of 150 hours at the Delhi Community Health Clinic. There she answered the phone, made appointments, filed documents, and scanned reports into computer files. She also worked on the weekends at that clinic doing somewhat of the same thing. She noted that when she was working her pain still increased in her left shoulder and that she had good days and bad days, depending on the nature of the work being done. She explained that the filing of documents caused her some problems because she was required to do it for two doctors.
- [106] In July, 2009, she got a job with Comcare. It is a private company which hires personal support workers (PSW) to care for its patients. When Ms. Slaght was hired, she initially worked as the "after hours on-call" receptionist. She worked from home and answered the phone from her residence from 7:00 p.m. each evening to 7:00 a.m. the next morning to deal with any emergency, new clients, or to replace a PSW who was not able to work that particular day. She did that for three days or four days per week. She recalled that

the hours increased when some staff left and at her busiest, she was working five to six night shifts per week.

- [107] In October 2009, a job opening arose at Comcare for a full-time day shift position. She accepted that position and now works in the office doing the same type of work. She coordinates the schedule of the PSWs and works with the Community Care Access Centre when a client is referred to their care. Most of her work is done by computer and over the telephone.
- [108] The defence did not seriously challenge the choice of alternative career made by the plaintiff. I find that she acted reasonably throughout. She obtained the assistance of a qualified vocational rehabilitation counsellor when she realized her injuries may cause her difficulty returning to her former employment. She chose to take the course at Fanshawe College which was only a 16 week full time course, compared to a 36 week course offered elsewhere. She clearly took reasonable steps to find a job as quickly as possible and started by working from home with Comcare on night shifts. Within a reasonably short period of time, she was granted a daytime job as a co-ordinator with Comcare where she has continued working to the time of trial. I find on the evidence that the plaintiff has followed a reasonable course of action to mitigate her damages by finding a job that is within her physical limitations.
- [109] She presently earns a salary of \$27,000.00 per year. She likes her job and feels that she is able to do it because it is within her physical limits. She acknowledged that she still takes pain medication some days because just the daily functions of the job cause her pain.
- [110] Ms. Slaght testified that she hopes to continue to do this job to the age of 65. She felt that if she could not physically do this job, there are probably not a lot of jobs in the workplace that she would be able to do. She stated that she likes the job and likes dealing with the people she has interaction with in that capacity.
- [111] She recalled that prior to the motor vehicle accident she had a pretty active social life. Aside from her two jobs, she used to like going to the gymnasium to work out and liked the way that she looked. She enjoyed social functions, including golf and the social aspect of the game.

Plaintiff's Expert's Calculations

- [112] The plaintiff called Roneld Smith, CA, IFA, CIP as an expert witness on the issue of the various economic losses suffered by the plaintiff as a result of the motor vehicle accident.
- [113] He calculated the losses based on the wage loss commencing one week after the accident. He was asked to assume that but for the accident, the plaintiff would have continued to work at Atlas and Curves to age 65. He was advised (and it was not disputed in this trial) that the plaintiff's long term income disability benefits stopped on June 19, 2009 at which point the plaintiff had received disability and income replacement benefits in excess of 80 per cent of her net after-tax income. He was instructed not give a credit to the defendant

for the amount of excess benefits received above 80 per cent of the net income loss. Counsel agreed at trial that such a credit should be given.

- [114] He found that the short term loss of income suffered by the plaintiff from June 20, 2009 to May 9, 2010 (the day before the commencement of trial) was \$6,500.00. Those calculations are found at tab 2 of his amended report which was entered as exhibit 5. I accept those calculations and the assumptions made to found them. Ms. Joynt, the defendant's expert, did not seriously disagree with those figures.
- [115] He estimated the pension loss suffered by the plaintiff to be \$11,000.00 if she works to age 65. In light of my finding that she would probably have worked at Atlas to the age of 65, the plaintiff shall have judgment for that amount. Again, that figure was not seriously challenged by Ms. Joynt.
- [116] The real difference between Mr. Smith's calculations and the defendants' expert related to the discount rate to be applied to calculate the future loss of income after the 15th year. The experts were in general agreement on how to discount the first 15 years of the future loss period. However, they applied two distinctly different interpretations on how to discount losses from year 16 onward.
- [117] Mr. Smith assumed that all losses from year 16 onwards are to be discounted at the formula rate for the first 15 years and at 2.5% for years thereafter.
- [118] Ms. Joynt's approach assumed that all losses from year 16 onwards are to be discounted at the rate of 2.5% per year from the date of trial.

Defendant's Expert's Calculations

- [119] Melissa Joynt is a chartered accountant employed with the firm of Matson, Driscoll and Damico Ltd., forensic accountants, which is a boutique firm specializing in economic damage calculations. She has participated in over 1500 economic loss calculations since she started as a manager with that firm in 2001. She prepared a report laying out her calculation of the plaintiff's short term wage loss, her pension loss and three scenarios estimating the plaintiff's future loss of income.
- [120] Her report, like that of Mr. Smith's, was detailed and helpfully organized into various charts to assist the court in following the witness' evidence with respect to the calculations to be made on the various components of loss.
- [121] She stated that her short term loss of income calculations and pension loss calculations are virtually the same as those of the plaintiff's expert, Mr. Smith. The major difference in their reports relates to the future loss of income, and in particular, the discount rates to be applied to calculate the future loss of income. The other major factor is the discount to be applied to such future loss of income, if any, for workplace participation.
- [122] In her report (exhibit 12), she noted that Mr. Smith's report interpreted Rule 53.09(1) in a manner different from her understanding of the legislation. She started with the

observation that the discount rate prescribed by the Rule beyond year 15 is 2.5% (the long term rate). She understood this to mean that the discount factor to be applied to the payment in year 16 should be based on the 2.5% discount rate in each of the 16 years. Based on that prescribed rate, the discount factor for year 16 (before consideration of mortality) is 0.674. This means that if the annual loss is \$1,000.00, the value of the discounted annual loss for year 16 should be \$674.00.

Application of Rule 53.09(1)(a) and (b) (the Discount Rate re Future Income Loss)

[123] I find that the appropriate method to apply Rule 53.09(1)(a) and (b) is that articulated by the plaintiff's expert Mr. Smith. In my view the rule is clear. During the first 15 years after the accident, the real discount rate applied in each year must be in accordance with Rule 53.09(1)(a). The experts agreed on that issue.

[124] I find that for the time beyond 15 years, Rule 53.09(1)(b) is clear. It states:

The discount rate to be used in determining the amount of an award in respect of future pecuniary losses, to the extent that it reflects the difference between estimated investment and price inflation rates is,

(a) for the 15 year period that follows the start of the trial.....

(b) for any later period covered by the award, 2.5 per cent per year.

[125] If the legislature had intended that payments occurring in years beyond the 15th year should be subject to a discount factor calculated using the 2.5 per cent discount rate in every year, it easily could have passed such legislation. To accept the position of the defendant's expert in this case, would indeed create an anomalous result. Mr. Smith highlighted this in his rebuttal report (exhibit 7) when he gave an example to illustrate his point. It is reproduced for ease of understanding.

“Using the MD&D methodology, you would obtain the following present values as today's date based upon a 0.75% and 2.50% discount rates:

Scenario 1: Assume that \$1,000,000.00 was payable at the end of 15 years from today (discounted at 0.75%).

Scenario 2: Assume that the same \$1,000,000.00 was payable 15 years plus a day from today (discounted at 2.5%).

For Scenario 1, today's present value would be approximately \$894,000 whereas for Scenario 2, it would be \$690,500.00.

It is not logical that a one day difference in the due date of a \$1,000,000 payment can yield a difference of almost \$203,500 in the above scenarios.

[126] I concur in his analysis. Mr. Smith concluded that the rule does not say that amounts that arise in the period before or after the 15 year period are discounted in a particular way. He contends that the yearly real discount rates to be applied during those periods are calculated in a certain way.

[127] To apply the rule as Ms. Joynt has suggested, the Rule would have to specify that certain future payments should be discounted at a given rate rather specifying discount rates to be used in specific time periods. If the legislature intended the result urged by Ms. Joynt, the legislation should have clearly stated so, possibly in terms such as these:

Any future payment that occurs beyond the 15th year following the date of trial is to be discounted at the real discount rate of 2.5% per annum for each year between the date of trial and the date the future payment is to be made.”

Such is not the wording of Rule 53.09(1).

[128] I have been referred to the decision of Lauers J. in *Greenhalgh et al v. The Corporation for the Township of Douro-Drummer and Henry Weidner*, 2009 CanLII 71014. In that decision, the court accepted the analysis offered by an expert from Matson, Driscoll and Damico Ltd. Tendered on behalf of the defence which applied the same methodology for calculating payments after year 15 that Ms. Joynt presented in her evidence before this court. Unfortunately, the plaintiff’s expert in that case was not called in reply to explain the rationale for his approach. As a result, the learned judge was not given the explanation so clearly articulated in this case by Mr. Smith on behalf of the plaintiff. In the circumstances, I respectfully do not agree with the approach applied by Lauers J. in the *Greenhalgh* case.

Analysis

[129] Mr. Perrott indicated that the company had not had any layoffs and he was not aware of any that were imminent. Ms. Slaght testified that the work at that factory had been modified to accommodate her limitations after the accident and there were positions that were not as physically demanding as others. Mr. Perrott, who had been her supervisor for approximately 5 years, stated that the plaintiff was a willing worker and the type of employee that an employer wants to keep. Her performance reviews reflect those comments. Furthermore, Ms. Slaght financially required the job to make ends meet. She had the incentive to continue working as long as possible to maximize her pension entitlement by having her employer match her annual contributions. While some of the work at Atlas was physical and demanding, Gregory Perrott indicated that there were repetitive jobs available which did not involve heavy lifting or pushing.

[130] I find that but for the accident, the Plaintiff would have continued to work at Atlas to age 65. In my view, the appropriate workforce participation rates should be applied to the future income loss calculation. While I recognize its application is to be used with discretion, it would be unfair to the defendants not to factor in those events which may shorten the anticipated working career of the plaintiff.

- [131] I find that but for the accident, the Plaintiff would have continued her employment at Curves to the end of the month when she attains the age of 60, as she clearly needed the money to pay for her outstanding mortgage and to cover the costs of day to day living. She enjoyed the work at Curves because it permitted her to work out and maintain herself in good physical shape, while earning a salary. Tinie Sandham, who owns a Curves franchise in Simcoe, stated that she likes to have staff members who represent all age groups and that she currently has an employee who is in her sixties. In making this decision, I have considered exhibit 10 which is a 2008 Government of Canada publication entitled Working in Canada. It considers factors relating to employees who are program leaders and instructors in Recreation, Sport and Fitness in the Hamilton-Brantford area including Simcoe. At page 2, it notes that the average age of retirement of workers is 62. Ms. Joynt readily agreed that she had not seen this report when she made her estimation that the plaintiff would have retired at age 55 from Curves.
- [132] I find that as Ms. Slaght aged, it is reasonably probable that working two jobs would become physically more difficult and less necessary as her economic situation gradually improved. I find that but for the accident, Ms. Slaght would have continued working two to three shifts per week and approximately 25 hours every two weeks at Curves. She would have continued to be paid at the minimum wage rate to the date of her retirement. She also would have been able to use the facilities at Curves for her personal training which translates into a free annual membership as an additional component of her income.
- [133] I find that the plaintiff will have to work at Comcare until the age of 65. Her present salary of approximately \$27,000 is not very significant for someone who has a mortgage to pay and a home to maintain. I am reinforced in that decision by Appendix A to the Rebuttal Report of Mr. Smith (Ex. 7), where it is noted that women employed in knowledge based jobs which impose less stress on the body are able to work longer. While the plaintiff's present job is stressful mentally, it does not have the physical demands associated with her employment at Atlas or at Curves.
- [134] When working at Atlas, she was earning more than at Comcare and still felt that she had to work a second job at Curves.
- [135] Based on the evidence of her current supervisor Kelly Spedding, I do not find that the plaintiff will be promoted beyond her current position as a job co-ordinator. She candidly acknowledged that based on what she has observed while working with the plaintiff for almost a year, the plaintiff does not have what it takes to become a team leader at this time because of her lack of motivation and confidence. In her cross examination, she agreed that she and the plaintiff had discussed the possibility of the plaintiff becoming a team leader and the plaintiff had indicated to her that she had no interest in it. While she agreed the plaintiff may change her mind in the future, Ms. Spedding did not see her being that type of personality as she seemed to lack confidence. She did not find that the plaintiff exhibited the ability to multi-task. When giving some of the answers relating to the plaintiff's career path, Ms. Spedding paused and seemed almost reluctant to give an answer that may personally hurt the plaintiff because of its

negative connotation. Nevertheless, she did so and I was impressed by her candid testimony and I accept it.

- [136] Ms. Spedding testified that the salary range for a client co-ordinator is \$27,000.00 to \$32,500.00 per year. To get to the top level of pay for that job, the supervisory staff looks at the number of years of service, performance appraisals, and the courses the employee has taken outside work to improve job performance. She testified that by five years, the employee should have been able to work herself up to the top salary in that range.
- [137] I find that the plaintiff is motivated to work hard and do her best, despite several of the limitations clearly expressed by Ms. Spedding. I find that the plaintiff's salary at Comcare will increase to \$28,500.00 by October, 2011 and by one thousand dollars annually each year thereafter until it reaches the maximum of \$32,500 in 2016.
- [138] By that time, she will be in her early to mid 50's and I find that she will continue to work at that position at the top range of a co-ordinator's salary to her retirement at the age of 65.
- [139] Ms. Spedding further testified that Comcare previously had a profit sharing plan for its employees which is going to be terminated. In 2011, it will be replaced by a performance bonus based on how the particular branch office is performing and based on the particular performance review of each employee. Ms. Spedding testified that she felt that the plaintiff met her expectations in her job performance review and that that was part of the eligibility for full participation in the annual performance bonus system. Ms. Spedding did not know what the percentage bonus could possibly be under this new program. I have no information from the parties on how this performance bonus will work or how much it will be.
- [140] The Plaintiff is entitled to compensation for her future loss of income. I disagree with Mr. Smith that no contingencies should be deducted for workplace participation because the plaintiff may well have worked beyond 65 in either of her jobs at Atlas or Curves. I do not find, based on her health prior to this accident and the fact she was working in a factory/warehouse setting at Atlas that it was reasonably possible or probable that she would have worked beyond age 65.
- [141] I would ask the parties to have the experts recalculate the loss of future income based on my findings in these reasons so that I can write an addendum to this judgment when I receive those calculations. If evidence of the performance bonus formula to be implemented by Comcare in 2011 is available, it should be included in the future income loss calculation. Otherwise, the sum of \$1,000.00 per year should be included as a best estimate of the plaintiff's annual performance bonus under the new performance bonus system to be implemented by Comcare. If the parties are able to determine the formula for that plan and wish to lead further evidence on that point before January 14, 2011, the Trial Co-ordinator can be contacted and a time set to receive that evidence. Otherwise, the figure of \$1,000.00 is to be applied to determine the plaintiff's future loss of income as otherwise directed in this judgment.

- [142] It is further ordered that the defendant is to be given credit against sums owing under this judgment for income replacement benefits received by the plaintiff in excess of 80% of her net income loss sustained to the date of trial which commenced May 10, 2010.

Loss of Competitive Advantage

- [143] Counsel for the plaintiff also submitted that the court could award damages to the plaintiff for loss of competitive advantage in the workplace caused by her injuries. I have considered that issue and find that whatever loss she may sustain in that respect is already reflected in the award for future loss of income. In the circumstances, the claim is dismissed.

Threshold Issue:

- [144] I find that the plaintiff has suffered a permanent impairment of an important bodily function that is serious, as defined under section 267.5(3) of the *Insurance Act*. Dr. Martin described her as having chronic pain in the left shoulder with limitations in her ability to lift above her shoulder. Dr. Johnston wrote in his Attending Physician's Statement of Disability of August 19th, 2007 that the plaintiff was suffering chronic pain and restricted rotation of movement in her left shoulder. In a subsequent letter dated December 11, 2007, he wrote that Ms. Slaght had a significant impairment in the mobility of her left shoulder with increased pain on abduction and internal and external rotation. He noted some wasting of the left shoulder girdle. He opined that the complication prolonging the disability was the suspected development of adhesions within the shoulder girdle combined with the weakness occasioned by the reduced muscle bulk.
- [145] Because of this injury sustained in the motor vehicle accident of December 22, 2004, the plaintiff was not able to continue with her former employment at Atlas Hydraulics and Curves. I find that efforts were made at Atlas to try to accommodate her impairment, but to no avail. She continues to suffer chronic pain and depressive reaction to the limitations placed on her former active and physical lifestyle. The pain has been continuous since the accident, subject to some relief from medication or when cortisone or botox injections occur, and it cannot reasonably be expected to substantially improve.

General Damages:

- [146] At the time of the accident, Ms. Slaght was an active 42 year old lady, who unfortunately was on the cusp of a marital breakup. She held down two jobs which she indicated she liked. Her performance evaluations at Atlas were excellent and she enjoyed her work at Curves. She was active in sports with a special passion for baseball and golf. She had a nice group of friends in her active social life, which was important to her as she went through the process of separation.
- [147] This motor vehicle accident occurred almost five years ago. At the time of trial, she was still in pain and experiencing significant limitation in the use of her dominant left arm. She continues to take pain medication for symptoms. Her lifestyle has changed and she has become more withdrawn and does not socialize in the same manner as before. Her

friend Carrie Patrick described Ms. Slaght as doing “a 360” after her injury in the 2004 motor vehicle accident. She found that despite making plans to go out, the plaintiff would cancel. To her, it was like the plaintiff became a recluse and stayed away from everyone. Ms. Patrick said her friend has never gotten back to being the same as before the accident. Today, she is more moody, frustrated and does a lot of complaining. Physically, she has gained a lot of weight since the accident. Ms. Patrick described the plaintiff as a very attractive woman before the motor vehicle accident.

- [148] At the end of her examination in chief, Ms. Slaght tearfully stated that she did not feel good about herself. She said that she felt useless and did not want to socialize much because she felt she pulled others down with her depressed attitude. She expressed sadness that she is not able to golf any longer and that all her friends are doing what she used to like to do. As I heard her testify, I found her comments to be sincere and truthful. I did not feel that she was trying to overstate her emotional condition.
- [149] Dr. Gouws noted that the plaintiff agreed that her demeanour had changed and she had increasingly isolated herself and become more angry. When he took a valid Personality Inventory Assessment in 2008, he noted that the plaintiff reported some symptoms of depression and anxiety and stress. However, stated that his most important finding was the negative self interpretation she presented in the form of a lack of self confidence and being very self critical. She reported that on the anger scale, she could be verbally aggressive with relatively little provocation. Interestingly, the evidence of Ms. Spedding independently confirmed these findings when she assessed the workplace performance of Ms. Slaght at Comcare. She noted that the plaintiff seems to lack self confidence and can let things build up before she lets them explode.
- [150] The plaintiff has given up golf and baseball. She has been forced to change careers and to re-educate herself to accommodate her ongoing disability. She will be forced to live with this the rest of her life. I note that the average life expectancy of a Canadian female is about 82 years of age, which probably means that the plaintiff will have to live with her physical limitations and her chronic pain for approximately 40 years.
- [151] The plaintiff has acted reasonably throughout in an effort to mitigate her damages. She was fully compliant in the physiotherapy recommended for her. She has taken cortisone shots to try to reduce her pain to permit her to function in her former jobs. She finally underwent surgery with mixed results. She has been taking botox injections to reduce her ongoing pain and to maximize function of her left shoulder. With the guidance of a vocational assessment counsellor and on the advice of her family doctor, she reluctantly decided to change careers and went to Fanshawe College to improve her skills to make herself eligible for a medical administration position. She sought and obtained employment in that field and has successfully maintained it.
- [152] I find that the plaintiff has suffered a significant injury and an ensuing life-long disability as a result of the motor vehicle accident.

- [153] I assess the plaintiff's general damages for pain, suffering and loss of enjoyment of life at \$110,000.00, as suggested by counsel for the plaintiff.

Conclusion:

- [154] The plaintiff shall have judgment against the defendants for general damages for pain and suffering and loss of enjoyment of life in the amount of \$110,000.00 plus pre judgment interest in accordance with the Courts of Justice Act.
- [155] The plaintiff shall have judgement against the defendants for her future loss of income in an amount to be calculated by the parties' experts and submitted to the Court for further consideration and finalization of judgment on or before January 14, 2011.
- [156] The plaintiff shall have judgment against the defendants for her loss of pension benefits in the sum of \$11,000.00.
- [157] The plaintiff shall have judgment against the defendants for her short term loss of income in the amount of \$6,500.00.
- [158] It is further ordered that the defendants are to be given credit against sums owing under this judgment for income replacement benefits received by the plaintiff in excess of 80% of her net income loss sustained to the date of trial which commenced May 10, 2010.
- [159] In the event that counsel are unable to agree on this issue of costs, counsel may file brief written submissions with respect to the issue of costs, including copies of any Offers to Settle under Rule 49. A full costs summary shall be provided pursuant to Rule 57 with copies of dockets, if available. Such submissions shall be submitted at the time that the reports of the experts with respect to the future loss of income are submitted to the court.

Turnbull, J.

Released: December 9, 2010

CITATION: Slaght v. Phillips and Wicaartz, 2010 ONSC 6464
COURT FILE NO.: CV-07-109

ONTARIO

SUPERIOR COURT OF JUSTICE

BETWEEN:

Deborah Slaght, Plaintiff

– and –

Robert B. Phillips and Peter Wicaart,

Defendants

REASONS FOR JUDGMENT

The Honourable Mr. Justice J. Turnbull

Released: December 9, 2010